CARECONNECT INDIA

Fresh MVP Developer Brief — v1.0

Aged Care & Parent Care Discovery Platform

Confidential — Internal Use Only

IMPORTANT: This is a greenfield developer brief. The existing portal/codebase is NOT the implementation baseline. Developers should create a clean, maintainable MVP from scratch while preserving the future product architecture described in this document.

Market: Pan-India, city-aware | Test Bed: Vizag (Visakhapatnam) | Initial Wave: Bangalore, Hyderabad, Chennai, Kochi, Coimbatore, Pune

Recommended stack: Next.js + TypeScript + Supabase + Vercel

1. PRODUCT VISION

CareConnect India will become a trusted platform for families to discover, compare, arrange and ultimately manage care for ageing parents and seniors in India.

The MVP is deliberately narrow: it is a trusted discovery and lead-generation marketplace. The long-term product is much larger.

Stage	Product
MVP	Find and compare aged-care providers → submit structured care request → provider receives lead.
Phase 2	Family and parent profiles → care needs → care plans → visits → updates → documents.
Phase 3	One-stop marketplace → caregivers, nursing, doctors, physiotherapy, diagnostics, pharmacy, equipment, emergency and senior living.
Future	AI-assisted matching, care summaries, alerts and care intelligence.

Long-term positioning: “CareConnect India — India’s trusted platform for caring for ageing parents.”

NRI proposition: “Your parents are in India. Their care doesn’t have to be.”

2. GREENFIELD DEVELOPMENT PRINCIPLE

The previous portal was created by another developer and should not be treated as the technical foundation for this project. The new team should assume a clean-slate implementation.

- Do not copy legacy application structure, components or database design unless explicitly reviewed and approved.
- Create a clean Git repository and documented branching/release strategy.
- Create separate development, staging/preview and production environments.
- Define database migrations from the beginning.
- Use environment variables/secrets management.
- Document architecture, setup, deployment and operational procedures.
- Use automated linting, formatting, type checking and tests.
- Build reusable components and configuration-driven city/service structures.
- Design the MVP so future family/care-management functionality can be added without a rewrite.

The goal is not to reproduce the old portal. The goal is to build the correct product from first principles.

3. PROBLEM TO SOLVE

Families often struggle to identify trustworthy aged-care providers, understand which type of service is appropriate, compare alternatives and contact providers quickly. This problem is particularly acute for adult children living away from their parents, including NRIs.

CareConnect should solve discovery and trust first. It should not attempt to become a clinical system or full care-management platform in MVP.

4. TARGET USERS

User	MVP job-to-be-done
NRI adult child	Find trustworthy care for parents in an Indian city from overseas.
India-based adult child	Find, compare and contact aged-care providers quickly.
Family member	Submit a care requirement and receive provider responses.
Care provider/agency	Create a credible listing and receive qualified enquiries.
CareConnect admin	Verify providers, manage cities/listings and monitor enquiries.

Future users: senior/care recipient, individual caregiver, healthcare professional, hospital/discharge team, insurer, senior-living provider and corporate partner.

5. MVP SUCCESS DEFINITION

The MVP is successful when a family can discover a real provider in an active city and submit a meaningful care request that reaches an approved provider reliably, while the provider can manage its listing and receive measurable leads.

Outcome	MVP target
Active launch cities	Vizag test bed followed by selected Wave 1 cities
Real supply	At least 20 approved providers in the first actively marketed city; target 50+ across Wave 1 at launch
Provider onboarding	Self-serve registration + admin approval
Consumer discovery	City + service + area/language/verification filters
Lead generation	Structured care request + email/WhatsApp provider alert
Provider monetisation	Free / Standard / Premium listing tiers
SEO	Indexable provider profiles and city/service pages
Mobile	Responsive consumer experience

6. MVP SCOPE — BUILD NOW

6.1 Consumer Website

- Homepage.
- City selector: manual selection plus location-aware suggestion where practical.
- Service/category search.
- Search results page.
- Filters: service, area, language, verification status and listing tier.
- Provider profile page.
- Structured care request/enquiry form.
- Family confirmation after enquiry.
- NRI landing page.
- About.
- How It Works.
- Contact.
- Privacy Policy.

- Terms / disclaimer pages.

6.2 Provider Portal

- Provider sign-up/login.
- Multi-step provider registration.
- Provider application status.
- Profile creation/editing.
- Services offered.
- Areas covered.
- Languages.
- Contact information.
- Pricing band.
- Established year and staff-size range.
- Verification information.
- Logo upload.
- Lead inbox.
- Basic analytics.
- Listing tier/subscription status.
- Subscription checkout for paid tiers.

6.3 Admin

A custom admin portal is not required for the first MVP. Supabase Studio can be used initially, provided operational controls and permissions are properly configured.

- Approve/reject provider applications.
- Activate/deactivate providers.
- Activate/deactivate cities.
- Set verification status.
- Manage listing tier.
- Review enquiries.
- Monitor lead delivery.
- Manage service/category lookup data.
- Manage basic provider metadata.

7. CARE SERVICE TAXONOMY

MVP should expose a manageable subset while the underlying taxonomy is extensible.

Category	MVP examples
Home Care	Caregiver, attendant, nursing, day care, 24×7 care
Companionship	Companion / senior support
Rehabilitation	Physiotherapy
Cognitive Care	Dementia care
Medical	Geriatric doctor / physician
Palliative	Palliative care
Residential	Senior living / assisted living
Future	Diagnostics, pharmacy, equipment, ambulance, occupational therapy, speech therapy, etc.

8. CONSUMER JOURNEY

1. User selects or searches for a city.
2. User chooses a service or describes the care requirement.
3. System displays relevant providers.
4. User filters by area, language, verification and tier.

5. User opens provider profile.
6. User submits a structured care request.
7. System stores the enquiry and sends it to the provider.
8. Family receives confirmation.
9. Provider receives lead by email and, for eligible tiers, WhatsApp.
10. Provider follows up directly.

MVP does not complete the care-service transaction. CareConnect is facilitating discovery and lead generation.

9. STRUCTURED CARE REQUEST

This is a key improvement over a generic contact form. The MVP should capture enough information to make the lead useful to a provider.

Field	Required?
Family name	Yes
Phone	Yes
Email	Optional
Care recipient age	Recommended
Care recipient gender	Optional
City	Yes
Area/locality	Recommended
Service needed	Yes
Care requirement	Yes
Mobility level	Recommended
Hours required	Recommended
Start date	Recommended
Duration	Optional
Preferred language	Optional
Preferred caregiver gender	Optional
Budget band	Optional
Urgency	Recommended
Additional message	Optional

Do not collect detailed medical/clinical history in MVP unless there is a clear, reviewed requirement. Keep the initial data collection proportionate to the discovery/lead purpose.

10. PROVIDER PROFILE

Field	Requirement
Provider name	Required
Slug	Unique, SEO-friendly
Description	Up to ~500 characters for summary; longer detail can be added later
City	Required
Areas covered	Required/recommended
Service categories	Required
Languages	Recommended
Phone/email	Required
Website	Optional
Address	Optional/public display configurable
Pricing range	Budget / Mid / Premium
Established year	Optional
Staff size	Optional
Verification status	Admin-controlled
Listing tier	Free / Standard / Premium
Logo	Optional
Operating hours	Future-ready field

Emergency/24×7 availability	Future-ready field

11. TRUST & VERIFICATION — MVP

Trust is a core product principle, but the MVP should not pretend that a provider is fully vetted merely because an admin toggled one generic badge.

Implement a basic verification status with room to expand.

Status	Meaning
Unverified	Provider application accepted/listed but verification not completed.
Business Verified	Basic business/provider information checked by CareConnect.
Credential Verified	Relevant credentials checked where applicable.
Background Check	Background/police check status where applicable and available.
CareConnect Verified	A defined combination of checks has been completed according to CareConnect policy.

Only show a badge when its definition is documented. Verification evidence and expiry/review dates should be stored internally.

12. LISTING TIERS

Feature	Free	Standard ₹1,999/mo	Premium ₹4,999/mo
Listed in search	Yes	Yes — boosted	Yes — top placement
Public profile	Yes	Yes	Yes
Lead delivery	No	Email	Email + WhatsApp
Verified badge eligibility	No	Yes	Yes
Analytics	No	Yes	Yes
Logo	No	Yes	Yes
Highlighted listing	No	No	Yes

Pricing is an initial hypothesis. Review after real provider and lead data. Do not hard-code commercial assumptions throughout the application.

13. NRI EXPERIENCE — MVP

NRI acquisition is a primary product hypothesis, not a separate product.

- Dedicated NRI landing page.
- Clear city/service entry point.
- Messaging focused on arranging care remotely.
- International-friendly phone/email capture.
- WhatsApp communication where appropriate.
- SEO landing pages for high-intent searches such as care for parents in specific cities.
- Future-ready family account architecture.

Future NRI features: parent profile, family dashboard, care plan, payments, visit updates, documents, alerts and sibling access.

14. SEARCH & SEO

SEO is a core acquisition channel.

- Server-rendered/ISR provider profile pages.
- Unique metadata and canonical URLs.

- Structured data where appropriate.
- XML sitemap.
- Robots.txt.
- Open Graph/social metadata.
- City/service landing pages.
- Provider profile indexing only when approved and active.
- Paginated search with crawl/index strategy defined by developers.
- Fast mobile page performance.

Examples of future SEO pages: Home Care in Hyderabad, Elderly Care in Bangalore, Dementia Care in Chennai, Physiotherapy for Seniors in Kochi, 24×7 Caregiver in Coimbatore.

15. TECHNICAL ARCHITECTURE — GREENFIELD

Layer	Technology
Frontend	Next.js App Router
Language	TypeScript
UI	Tailwind CSS + reusable component system
Database	Supabase PostgreSQL
Authentication	Clerk or Supabase Auth — final choice before implementation
Storage	Supabase Storage
Hosting	Vercel
Email	Resend or equivalent transactional email provider
WhatsApp	WhatsApp Business API provider such as 360dialog; isolate integration behind service layer
Payments	Stripe initially for provider subscriptions; payment abstraction for future India consumer payments
Analytics	Privacy-conscious product analytics + server-side event tracking as appropriate

The previous implementation used a particular combination of services. The new developer may recommend alternatives, but any change must be documented and approved before implementation.

16. ARCHITECTURE RULES

- Never expose service-role database keys to the browser.
- Use Row Level Security for all user/provider-owned data.
- Use migrations for schema changes.
- Validate all API input server-side.
- Rate-limit public enquiry endpoints.
- Protect against spam/bot submissions.
- Use secure file-upload rules.
- Use typed API contracts.
- Separate public provider data from private operational data.
- Do not expose family contact information through public APIs.
- Maintain audit logs for admin changes.
- Use feature flags/configuration for future functionality where useful.
- Build responsive-first.
- Prefer server components/server-side data fetching for SEO-critical pages.

17. DATABASE — MVP

Table	Purpose / key fields
users	Application identity/roles where required
providers	Provider profile, ownership, services, city, verification, tier, status

provider_applications	Registration/application workflow and approval status
cities	Name, slug, state, wave, active flag, coordinates
service_categories	Hierarchical category definitions
services	Individual service definitions
provider_services	Many-to-many provider/service relationship
enquiries	Family request, provider, service, status, delivery metadata
lead_deliveries	Email/WhatsApp delivery attempts and status
subscriptions	Provider tier, billing identifiers, status
provider_analytics	Aggregated views/enquiries or event references
audit_logs	Admin/security-sensitive actions
consents	Consent records where required

Avoid storing sensitive health information in the MVP unless necessary. If future care-management data is introduced, create separate care-domain tables with stricter access controls.

18. FUTURE DATA MODEL — DO NOT BUILD NOW

- families
- family_members
- care_recipients
- care_assessments
- care_needs
- care_plans
- care_tasks
- caregivers
- provider_staff
- healthcare_professionals
- care_requests
- matches
- bookings
- visits
- attendance
- care_notes
- medications
- appointments
- medical_documents
- payments
- commissions
- reviews
- ratings
- incidents
- emergency_events
- notifications

These entities should be considered in architecture discussions, not MVP implementation scope.

19. API — MVP

Method	Endpoint	Purpose
GET	/api/cities	Active cities + provider counts
GET	/api/services	Active service/category lookup
GET	/api/providers	Search/filter/paginate providers
GET	/api/providers/[slug]	Public provider profile
POST	/api/enquiries	Create structured family enquiry
POST	/api/provider/register	Create provider application
GET	/api/provider/me	Authenticated provider profile

PATCH	/api/provider/me	Update provider profile
GET	/api/provider/leads	Provider lead list
GET	/api/provider/analytics	Basic provider analytics
POST	/api/webhooks/stripe	Subscription events

API response shapes, validation schemas, authentication requirements, error codes and pagination rules must be documented before implementation.

20. NOT IN MVP

- Online booking for care services.
- Consumer payment for care services.
- In-app family/provider messaging.
- Native iOS/Android app.
- AI diagnosis or care recommendations.
- AI matching.
- Full family accounts.
- Longitudinal medical records.
- Medication management.
- IoT/remote monitoring.
- Automated emergency response.
- Individual caregiver marketplace.
- Complex review/rating system.
- Custom enterprise partner portals.
- Custom admin UI unless Supabase Studio becomes operationally insufficient.

21. FUTURE PRODUCT — BIG PICTURE

21.1 Phase 2 — Care Management

- Family account.
- Parent/care recipient profile.
- Care needs assessment.
- Care plan.
- Care tasks.
- Caregiver assignment.
- Visits and attendance.
- Daily care notes.
- Family updates.
- Appointments.
- Document vault.
- Notifications and alerts.

21.2 Phase 3 — One-Stop Marketplace

- Individual caregivers.
- Home nursing.
- Doctors.
- Physiotherapy and rehabilitation.
- Diagnostics.
- Pharmacy/medicine delivery.
- Medical equipment.
- Ambulance/emergency partners.
- Senior living.
- Assisted living.
- Respite care.

21.3 Future — Care Intelligence

- AI-assisted provider matching.
- AI-generated family summaries.
- Care-plan assistance.
- Trend/anomaly alerts.
- Provider quality intelligence.
- NRI care assistant.
- Natural-language family support.

AI must remain assistive. It should not independently diagnose or make high-impact clinical decisions.

22. FUTURE FAMILY DASHBOARD — REFERENCE

Area	Example
Parent	Mother — 74, Hyderabad
Today's care	Caregiver visit, medication, physiotherapy
People	Caregiver, doctor, physiotherapist
Updates	Daily care summary
Documents	Reports, prescriptions, discharge summaries
Appointments	Upcoming doctor/physio
Alerts	Missed visit, incident or configured notification
Family	Son, daughter, authorised family members

This is a future-state design reference, not an MVP screen requirement.

23. FUTURE CARE REQUEST / MATCHING

The long-term workflow should evolve from one-provider enquiry to a structured care request that can be matched against multiple suitable providers.

Example: “Need a 12-hour caregiver for a 78-year-old father in Hyderabad, starting Monday.”

Future system output: suitable providers ranked using transparent factors such as service capability, location, availability, verification, language and family preferences.

AI may assist in ranking later, but the MVP must not require AI.

24. FUTURE TRUST MODEL

- Identity verification.
- Business verification.
- Credential verification.
- Background checks.
- Training/certification.
- Insurance status.
- Verified reviews.
- Provider response reliability.
- Caregiver attendance reliability.
- Incident history.
- CareConnect quality score.

The platform should eventually distinguish verification facts from subjective ratings. A provider's commercial subscription must never automatically determine whether it is medically or operationally ‘trusted’.

25. PAYMENTS & REVENUE ROADMAP

MVP revenue: provider listing subscriptions.

Future revenue:

- Provider subscriptions.
- Qualified lead fees.
- Transaction commission.
- Premium family subscription.
- NRI managed-care subscription.
- Hospital/healthcare partnerships.
- Insurance/corporate partnerships.
- Senior-living and equipment referrals.

Keep billing logic modular. Commercial rules should be configurable rather than embedded in UI code.

26. SECURITY, PRIVACY & COMPLIANCE

- RLS for all protected tables.
- Least-privilege access.
- Secure authentication/session handling.
- Admin audit logging.
- Secure secrets management.
- Input validation and sanitisation.
- Rate limiting and abuse protection.
- Secure document storage when introduced.
- Consent management where required.
- Data retention/deletion process.
- Incident/breach response process.

The MVP should minimise health-related data collection. Before introducing clinical records, telemedicine, medical recommendations or large-scale health-data processing, obtain appropriate legal/privacy review for the exact Indian operating model.

27. DEVELOPMENT WORKFLOW

11. Confirm product scope and architecture.
12. Create Git repository and project documentation.
13. Create development/staging/production environments.
14. Implement database migrations and RLS.
15. Implement authentication and roles.
16. Implement provider onboarding.
17. Implement cities/services.
18. Implement public search/profile experience.
19. Implement structured enquiry and delivery.
20. Implement provider dashboard.
21. Implement subscriptions.
22. Implement analytics and monitoring.
23. Complete security/performance/SEO QA.
24. Seed real provider data.
25. Production launch.

28. SPRINT PLAN

Sprint	Primary deliverables
1 — Foundation	Repository, architecture, environments, CI, DB migrations, RLS, auth, design system, cities/services
2 — Provider Supply	Provider application, approval workflow, provider profile editor, verification status, storage
3 — Consumer Discovery	Homepage, city selection, service search, filters, provider profiles, SEO
4 — Lead Engine	Structured enquiry, email delivery, WhatsApp integration, lead status, family confirmation
5 — Monetisation	Free/Standard/Premium, checkout, subscription webhook, provider dashboard, analytics
6 — Launch Hardening	Real data, mobile QA, SEO QA, security, performance, monitoring, production deployment

Sprint durations should be estimated after technical discovery. Developers must not commit to fixed day counts without reviewing the greenfield implementation.

29. TESTING & QUALITY

- Unit tests for business logic.
- API validation tests.
- Authentication/authorisation tests.
- RLS tests.
- Provider registration end-to-end test.
- Enquiry delivery end-to-end test.
- Subscription/webhook tests.
- Mobile responsive testing.
- SEO/indexability checks.
- Accessibility baseline testing.
- Performance testing on representative mobile conditions.
- Security review before production.

30. MVP ACCEPTANCE CRITERIA

- A new provider can register without developer intervention.
- Admin can approve/reject provider applications.
- Approved providers appear only in active cities.
- Consumers can search by city and service.
- Filters work correctly and consistently.
- Provider profiles are server-rendered/indexable.
- Provider contact information is not exposed to unauthorised users.
- Family can submit a structured enquiry.
- Provider receives the lead reliably.
- Family receives confirmation.
- Lead delivery status is recorded.
- Provider can view leads in its dashboard.
- Paid listing checkout and subscription status work.
- RLS and authorisation are verified.
- No secrets are exposed client-side.
- Core pages work on mobile.
- At least the agreed number of real providers are live before marketing launch.

31. ANALYTICS & KPIs

Metric	Why it matters

Active providers	Supply health
Verified providers	Trust/supply quality
Provider profile views	Discovery demand
Enquiries	Core MVP conversion
Profile→enquiry rate	Marketplace quality
Lead delivery success	Operational reliability
Provider response time	Lead quality
Paid provider conversion	Revenue
Provider retention	Marketplace health
NRI traffic/enquiry conversion	Priority customer segment
Organic traffic	SEO engine

32. GO-TO-MARKET

32.1 Supply

- Start with Vizag as controlled test bed.
- Directly recruit and verify providers.
- Use introductory/free listing incentives.
- Use hospitals/discharge teams as referral channels.
- Expand only after supply and lead delivery work reliably.

32.2 Demand

- SEO for city/service intent.
- NRI landing pages.
- Australian/US/UK/Gulf/Canada diaspora communities.
- WhatsApp referral loops.
- Hospital/discharge referrals.
- Partnerships with senior-care organisations.

32.3 City sequence

Test Bed: Vizag. Wave 1: Bangalore, Hyderabad, Chennai, Kochi, Coimbatore, Pune. Wave 2: Ahmedabad, Chandigarh, Mysore, Jaipur. Wave 3: Mumbai and Delhi NCR.

33. PRODUCT GUARDRAILS

- Do not become a generic services directory.
- Do not make unverified medical claims.
- Do not imply provider verification is stronger than the actual checks performed.
- Do not collect unnecessary health data in MVP.
- Do not build mobile apps before the web marketplace proves demand.
- Do not build AI before structured data and operational workflows exist.
- Do not let paid listing tiers override trust/safety signals.
- Do not expand to many cities before supply quality is proven.
- Do not tightly couple the MVP to one vendor where a future marketplace/payment provider may need to change.

34. FINAL DEVELOPER HANDOFF

The development team should treat this document as the greenfield MVP specification and product roadmap.

The immediate build is NOT a rebuild of the previous portal. It is a clean implementation of the validated MVP concept:

City → Service → Provider → Trust → Structured Care Request → Lead → Provider Response

The team must simultaneously preserve the architectural path toward:

Discovery → Marketplace → Care Management → One-Stop Parent Care → AI-enabled Care Intelligence

The MVP must therefore be small, fast and commercially testable, but its data model, APIs, security model and component architecture must not create a dead end for the future platform.

APPENDIX A — RECOMMENDED MVP NAVIGATION

Public	Provider	Admin
Home	Dashboard	Providers
Search	Profile	Applications
Provider Profile	Leads	Cities
NRI Care	Subscription	Services
How It Works	Settings	Enquiries
About	—	Verification
Contact	—	Analytics

APPENDIX B — FUTURE PRODUCT NAVIGATION

Family	Provider/Caregiver	Admin/Partner
Dashboard	Dashboard	Provider management
Parents	Leads/requests	Verification
Care Plan	Bookings	Quality
Visits	Schedule	Incidents
Updates	Team/caregivers	Partners
Documents	Analytics	Marketplace
Appointments	Payments	Reporting

END OF CARECONNECT INDIA — FRESH MVP DEVELOPER BRIEF v1.0

Prepared as a greenfield specification based on the prior CareConnect v2.0/v3.0 product direction, with the existing portal explicitly excluded as the implementation baseline.